Case study: Liam

Condition: **Distal radius fracture – right fracture, closed reduction and percutaneous K-wire fixation**

23-year-old man · Outpatient hand therapy clinic, regional public hospital

Liam O'Connor is a 23-year-old right-handed man with a **right distal radius fracture** who attends the outpatient hand therapy clinic of a regional public hospital. Liam is a third-year apprentice electrician. Three weeks ago, he fell from a ladder at work, and his fracture was treated with closed reduction and percutaneous Kirschner wire (K-wire) fixation. The ends of the wires protrude through the skin on the back of his wrist and are protected by a thermoplastic forearm splint. He has an accepted WorkCover Queensland claim and lives with his girlfriend in a rented unit.

Before Liam arrives, his site supervisor phones the clinic asking for "a quick update on what he can do" so that Liam can be rostered on light duties next week, and asks for a WorkCover capacity form to be returned today. At Liam's last hand therapy review, two weeks ago, his pin sites were documented as clean and dry with good finger range of motion. The most recent orthopaedic letter describes his fracture position on X-ray as "satisfactory".

You have been asked to review Liam's splint and check his pin sites. Liam is cheerful and talkative. He asks for a waterproof cover "so I can get a surf in this weekend", wants to know when he can get back to the gym and mentions that it is his girlfriend's birthday dinner tonight. When you ask how the wrist is going, he says, "Yeah, mint. Barely notice it."

The splint straps are frayed and grubby, and the padding around the wires is stained yellow-green. When you remove the splint, there is a sour smell. The skin around two of the three pin sites is red and puffy, with the redness spreading about two centimetres from each wire, and thick, creamy discharge is crusted around one of them. The back of his hand is swollen and feels warmer than his left. Liam admits the wrist has been "throbbing a bit more" over the past three days and that last night he felt "hot and shivery, but that was probably the heater".

When you ask about the splint, Liam grins and says he has been taking it off at work because "it gets in the way". His boss has had him on "light stuff", which turns out to be pulling cable through ceilings and feeding conduit. "I mostly use my left," he says. He smokes about 15 cigarettes a day, "less since the accident". The orthopaedic registrar gave him a prescription at his last clinic visit, but he did not fill it: "Didn't reckon I needed it." He asks whether you could also fill in "the WorkCover bit" so his boss stops ringing.

There is no documentation of what the prescription was for, whether anyone has looked at the pin sites in the past two weeks or what pin-site care Liam was taught. The orthopaedic letter does not say when the wires are due to be removed or what Liam is permitted to lift, and you do not know whether his employer has received any written restrictions.

As you finish examining the wires, reception puts a call through: Liam's site supervisor is on the line again, asking whether Liam can "come straight back after this and finish the switchboard job". Liam is already pulling on his hoodie. "Just chuck the splint back on and I'll grab a cover on the way. I'll be right."

You are the occupational therapy student reviewing Liam's splint and pin sites and your clinical supervisor is not currently in the room.

Present Liam's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.

ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

I Introduction

≈ 20 s

Who you are, your role, who the client is and why you need to talk now.

S Situation

≈ 40 s

A brief but complete summary of what is happening and the immediate concern.

B Background

≈ 1 min

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

A Assessment

≈ 2 min 20 s

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

R Recommendation

≈ 2 min 40 s

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

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